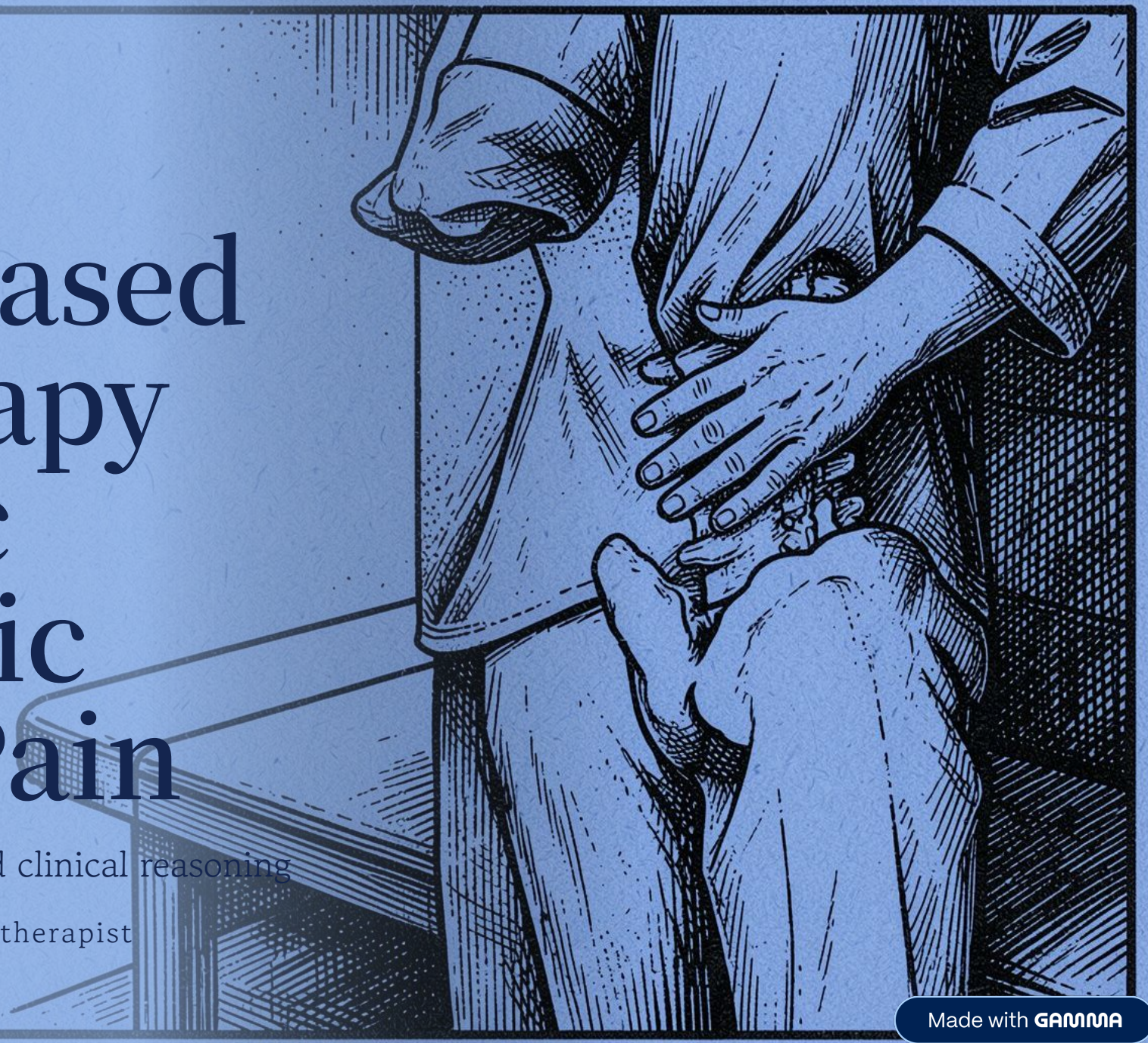
Evidence-Based Physiotherapy for Chronic Non-Specific Low Back Pain

Part 1 · Understanding, assessment and clinical reasoning

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PART 1 · ORIENTATION

What this first part helps you do

01 Classify the problem

Define chronic non-specific low back pain and separate it from specific pathology.

02 Screen and assess

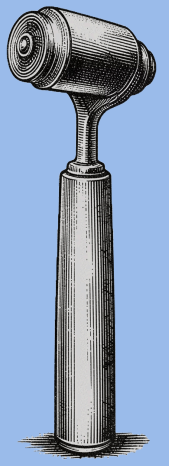
Screen for red flags, then build a focused history and examination.

03 Use imaging selectively

Use imaging selectively and explain findings without creating fear.

04 Plan the conversation

Communicate prognosis, activity and next steps through shared decisions.



Educational overview, not original research and not a substitute for individual assessment.

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CLASSIFICATION BEFORE INTERVENTION

Start with classification, not a label from a scan

NON-SPECIFIC LOW BACK PAIN

Symptoms without a clear serious structural or systemic cause after appropriate assessment.

SPECIFIC PATHOLOGY

Suspected fracture, infection, malignancy, inflammatory disease or significant neurological compromise.

> “Non-specific” does not mean imaginary or unimportant. It means the clinical pattern is not explained by one specific lesion.

Roman Urdu

Dard haqeeqi hai; naam na milna iska matlab yeh nahi ke masla jhoota hai.

Source note: WHO guideline for chronic primary low back pain, 2023; NICE NG59.

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FUNCTIONAL BURDEN

The burden is measured in life, not only in pain scores

The clinical problem becomes clearer when symptoms are connected to what the person can no longer do.

01

Symptoms

Pain, stiffness, poor sleep or low confidence.

02

Activity limits

Walking, lifting, self-care, study or work become harder.

03

Participation impact

Roles, routines and valued life activities are reduced.

Ask first: “What has this stopped you from doing?”

Impact is shaped by symptoms, confidence, environment, job demands and access to support.

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WHOLE-PERSON ASSESSMENT

Pain is real, and the whole context matters

The biopsychosocial model widens assessment and treatment options. It does not relocate pain into the mind.



Biological

Tissue sensitivity, sleep, deconditioning, comorbidity, neurological features



Psychological

Worry, low mood, fear of movement, self-efficacy, expectations



Social

Work demands, family roles, finances, healthcare access, cultural context

Source note: WHO guideline, 2023; NICE NG59.

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SAFETY SCREEN

Red flags change the pathway

Urgent concern

New bladder or bowel disturbance, saddle sensory change or rapidly progressive neurological deficit.

Consider serious pathology

Significant trauma or suspected fracture, infection, malignancy, inflammatory disease or unexplained systemic illness.

Interpret in context

Consider history, examination, risk factors, symptom change and clinical judgment.

Change the pathway

Do not reassure past a red flag. Escalate or refer using local pathways when serious pathology is suspected.

Source note: NICE NG59 recommendations 1.1.1 and related referral guidance.

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HISTORY TAKING

A focused history reveals the clinical problem

1. **Pattern:** Onset, duration, pattern, irritability and 24-hour behaviour.
2. **Function:** Sitting, standing, walking, lifting, sleep, work and valued activities.
3. **Clinical context:** Previous episodes, treatment response, medication, comorbidities and neurological symptoms.
4. **Meaning and goals:** Beliefs, concerns, stressors, confidence, barriers and the patient's goals.

Ask in plain language, then reflect back the patient's story before testing.



Roman Urdu: "Aap ke liye sab se mushkil kaam kaunsa hai?"

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EXAMINATION

Examine the person, not a single special test

01

Observe

Movement, posture, gait and symptom response.

02

Screen

Neurological function when history or symptoms indicate it.

03

Compare

Relevant hip movement and other contributors.

04

Test

A working hypothesis through functional or repeated tasks.

Use findings to test a working hypothesis; avoid over-reliance on isolated special tests.

Finish by explaining what the examination means and what it does not prove.

Source note: JOSPT/APTA clinical practice guidance for low back pain; NICE NG59.

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IMAGING

Imaging should answer a clinical question



- Do not routinely image uncomplicated low back pain in non-specialist settings when serious pathology is not suspected.
- Consider imaging when the result is likely to change management or when serious pathology is suspected.
- Common age-related findings may be incidental and do not automatically explain pain.
- Always correlate imaging with the history, examination, function and patient goals.

Scan par nazar aane wali har cheez dard ki wajah nahi hoti.

Source note: NICE NG59 recommendation 1.1.4; WHO guideline, 2023.

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PART 1 · TAKEAWAY

Prognosis is a conversation, not a promise

- Reassure without dismissing: validate symptoms, explain uncertainty and name a workable next step.
- Encourage appropriate activity and graded participation within the person’s tolerance and goals.
- Use shared decisions: agree what to monitor, when to review and when to escalate.
- Part 2 will move from reasoning to evidence-informed management and progression.

“Aap ka dard samjha ja raha hai; hum mil kar safe aur meaningful progress ka plan banayenge.”

Source note: WHO guideline, 2023; NICE NG59 shared decision-making and management recommendations.



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